

Breast Reconstruction Consultation: Assessment and Plan Template

Version 1.1, September 17, 2026. Built from the review of nine colleague notes across three providers, the ABPS 2026-2027 Booklet of Information requirements, the examiner rating items, and the user's own dictated sentences. Every paragraph is written in the user's voice. Brackets are dictated fields; the scribe leaves them blank if not stated so gaps are visible at sign-off. Delete any module that does not apply; never leave education or consent text for a modality the patient is not receiving.

Part 1. Style rules (apply to anything added later)

1. First person, past tense, for what you did: "I reviewed," "I explained," "I recommended."
2. The patient's response is its own sentence with the patient as subject: "Patient verbalized understanding that..." "Patient wishes to proceed."
3. Conditions carry an explicit then: "If [], then [], and in that case []."
4. Paragraphs open with a sequence marker where one fits: "After discussion of...," "At the conclusion of the consultation..."
5. Sentences close with purpose or contrast where it adds meaning: "in order to...," "rather than risk...," "as well as..."
6. Use the recurring nouns: surgical plan, surgical option, reconstructive goals, her concerns, a later stage, revision stage.
7. Plain sentences of 20 to 35 words, one idea each, in prose; the only list is inside the risk sentence.
8. No sentence may contradict the history, the exam, or another sentence in the plan.

Part 2. Fixed practice lines

- Bilateral DIEP: 6 to 8 hours under general anesthesia, at least 2 inpatient nights, 6 to 8 weeks to full activity.
 - Expander or implant reconstruction: 3 to 4 hours under general anesthesia, 23-hour observation, 4 to 6 weeks to full activity.
 - Expander plane is an intraoperative decision (paragraph in Part 4, module E).
 - For a grade III breast reconstructed with a flap, the skin pattern (Wise-pattern skin-reducing mastectomy versus skin-sparing mastectomy with a later mastopexy) is discussed with the breast surgeon before the surgical date is set (module M3).
 - Risks are listed in every note where a surgical plan is offered.
 - A1C is ordered only for diabetic patients.
 - Every stage is at least 3 months apart; the definitive stage after radiation is at least 6 months from the completion of radiation.
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Part 3. Assessment (numbered, declarative)

1. [Age]-year-old [pre/postmenopausal] woman with [side] breast [histology], [grade], cT[] ([] cm at [clock position], [] cm from the nipple) cN[], ER/PR [], HER2 [], genetics [negative/pending/not indicated], family history []. [Second lesion if multicentric.] Planned [unilateral/bilateral] [nipple-sparing/skin-sparing/skin-

reducing] mastectomy with [sentinel node biopsy/axillary dissection] by Dr. [] on [date]. [Contralateral mastectomy is risk-reducing by informed preference.]

2. Adjuvant therapy anticipated: [chemotherapy: none / neoadjuvant, last cycle (date), surgery 4 to 8 weeks after / adjuvant, to start within 30 to 60 days of surgery / pending Oncotype]; radiation [unlikely / possible / planned], based on [nodes, T stage, margins, response]; endocrine or targeted therapy []. Systemic decisions are per Dr. [medical oncology]; radiation per Dr. [radiation oncology].
3. Reconstruction candidacy: patient prefers [own tissue / implants / undecided] and a [same / smaller / larger] size. [Donor site: abdominal pinch () cm above and () cm below the umbilicus, scars (), estimated flap volume () mL per side against breast volume about () mL / abdomen unavailable because (), thigh and gluteal tissue ().] [Nipple-sparing: anatomically (favorable/borderline/not a candidate) with SN-N () cm, N-IMF () cm, ptosis grade (); oncologic eligibility per Dr. ().]
4. Perioperative risk: ASA [], Caprini [number], nicotine [never / quit (date) / current], BMI [], [diabetes with HbA1c () / no diabetes], anticoagulation [], hormone therapy [], medications [], allergies [], [MRSA history / prior radiation / thrombotic history].

Part 4. Plan (narrative, in order)

A. OPTIONS (EVERY NOTE)

I reviewed the options with the patient, which included no surgical reconstruction with a flat closure or external prosthesis, delayed reconstruction after cancer treatment, implant-based reconstruction done in a one or two stage manner, and autologous reconstruction as well. For each surgical option I explained how it could or could not address her concerns and reconstructive goals. [One sentence per option that was set aside and why: "Abdominal flap reconstruction is not possible because of her prior abdominoplasty." "Immediate implant reconstruction is not recommended because radiation is expected." "She is not interested in a flat closure or a prosthesis."]

B. RECOMMENDATION (EVERY NOTE)

After discussion of the surgical options, I recommended [procedure, laterality, staging], because [the two or three facts from the assessment that decide it: measurements, oncologic timing, radiation likelihood, donor site, patient goals]. Patient [agrees with this plan / elected () after discussion of ()].

C. RISKS (EVERY NOTE)

After discussion of the surgical options, I explained the risks of the surgery, specifically stating the risks of [insert the list from Part 5 for the chosen pathway, plus the patient-specific items], as well as the alternatives to the surgical plan and the expected benefits. I explained that exact symmetry is rare and difficult to achieve surgically, and that no promise of exact symmetry has been made nor should be expected. Patient verbalized understanding.

D. BACKUP PLAN (EVERY NOTE)

I explained that if [the mastectomy skin flaps are poorly perfused on ICG angiography], then [the expander will be left deflated or reconstruction delayed], and that if [], then []. [For flaps: "If the CT angiogram does not show adequate perforators, then the plan will change to a muscle-sparing TRAM or a thigh flap. If the flap is not viable during surgery, then a tissue expander will be placed as a staged alternative."] Patient verbalized understanding that the plan may change in the operating room in order to protect the skin envelope and the reconstruction.

E. EXPANDER PLANE (IMPLANT-BASED NOTES)

I explained that the expander plane will be decided in the operating room after the mastectomy. This decision is based on the thickness and quality of the mastectomy skin flaps on direct inspection, the perfusion of the skin flaps and nipples on ICG angiography, the amount of any poorly perfused skin that may need to be excised, the integrity of the pectoralis major after the mastectomy, the size and ptosis of the breast and the expected final implant volume, whether radiation is anticipated, and her own risk factors including BMI, nicotine exposure, diabetes, and prior radiation. I explained that well-perfused flaps of adequate thickness favor prepectoral placement with mesh in order to avoid animation deformity, and that thin or marginally perfused flaps favor subpectoral placement for muscle coverage, a lower initial fill, or a deflated expander. Patient verbalized understanding that either plane may be used.

F. RADIATION CONTINGENCY (EVERY NOTE WHERE RADIATION IS POSSIBLE)

Expander version. I explained that if the final pathology of the specimen demonstrates a positive node, then radiation becomes likely, and in that case the expander will stay in place through radiation and be exchanged at a later stage, at least 6 months from the completion of radiation. Patient verbalized understanding that the need for radiation can change her expander fill and timing, and that conversion to her own tissue at that stage would be discussed.

Flap version. I explained that if the final pathology of the specimen demonstrates a positive node, then radiation becomes likely, and in that case the flap will be radiated, which can cause firmness, shrinkage, and fat necrosis, and any revision will wait at least 6 months from the completion of radiation. Patient verbalized understanding.

Radiation already planned. I explained that radiation is planned after her mastectomy, and that in that case [the expander will be filled in the operating room to the volume agreed with Dr. (radiation oncology) with no fills after simulation / the reconstruction will be delayed until at least 6 months from the completion of radiation]. I explained that implant reconstruction after radiation carries a much higher risk of infection, capsular contracture, thinning of the skin, exposure, and removal, and that reconstruction with her own tissue tolerates radiation far better. Patient verbalized understanding.

G. STAGING AND TIMELINE (EVERY NOTE)

I explained that the reconstruction will involve [number] stages, each at least 3 months apart, with [exchange at about 3 months if no adjuvant treatment / exchange or conversion at least 6 months from the completion of radiation / revisions, nipple reconstruction, and contralateral symmetry at 3 months or later]. [If chemotherapy is planned after surgery: "I explained that chemotherapy is expected to start within 30 to 60 days of surgery, and that the first priority of the reconstruction is a healed wound so that treatment is not delayed."] The index operation is expected to take [hours] under general anesthesia with [23-hour observation / at least 2 inpatient nights] and a recovery of [4 to 6 / 6 to 8] weeks.

H. SAFETY AND COORDINATION (EVERY NOTE)

The surgery will be performed at [facility, accredited] under general anesthesia. Her Caprini score is [number], and she will receive sequential compression devices [and enoxaparin for () days]. [Nicotine, glycemic, anticoagulation, or MRSA line as applicable.] I will coordinate with Dr. [breast surgeon] regarding [incision, skin pattern, nipple decision, axilla], with Dr. [medical oncology] regarding [timing of systemic therapy], [with Dr. (radiation oncology) regarding volume and timing], [and with genetics regarding the pending result].

I. ORDERS (EVERY NOTE)

Routine labs [with PT and PTT for microsurgery], chest radiograph, ECG [per anesthesia], [A1C for diabetic patients], [CT angiogram abdomen and pelvis with 0.75 to 1 mm cuts and creatinine first], [echocardiogram per oncology], [MRSA screen], [anticoagulation clearance letter], [genetic testing].

J. FOLLOW-UP (EVERY NOTE)

I will see the patient in person for a preoperative visit at least one day before surgery on [date] in order to sign consent and mark. She will be seen immediately after surgery, in person within 30 days of surgery, [at the start and midpoint of radiation,] and at 90 days after surgery on [date] for postoperative photographs. [For radiated patients: "She will return at 6 months from the completion of radiation to schedule the definitive stage."]

K. CLOSING (EVERY NOTE)

At the conclusion of the consultation, the patient verbalized her understanding of the plan back to me and stated that all of her questions had been answered. Patient wishes to proceed. Preoperative photographs were taken with the patient's consent, and the patient signed the records and photograph consent for examination and certification purposes by The American Board of Plastic Surgery. We will obtain insurance authorization; coverage of all stages, contralateral symmetry, and prostheses under WHCRA was explained.

Part 5. Risk lists (insert into paragraph C)

General, every plan: bleeding, hematoma, seroma, infection, delayed wound healing and dehiscence, mastectomy skin flap necrosis, unfavorable scarring, asymmetry, contour irregularity, chronic pain, numbness of the chest and reconstructed breast, deep venous thrombosis and pulmonary embolism, anesthetic complications, need for revision or additional surgery, need for contralateral procedures, and dissatisfaction with the final result.

Implant-based, add: capsular contracture, implant rupture or leakage, malposition or rotation, rippling, animation deformity with subpectoral placement, implant exposure and removal, breast implant-associated anaplastic large cell lymphoma and squamous cell carcinoma, systemic symptoms reported with implants, the fact that implants are not lifetime devices and will likely need replacement, the need for MRI or ultrasound surveillance of silicone implants, and mesh-related infection or seroma. [Add: "I reviewed the FDA boxed warning and the Patient Decision Checklist, which will be signed at the preoperative visit."]

Autologous, add: partial or total flap loss with return to the operating room, fat necrosis, abdominal bulge or hernia, abdominal weakness, umbilical problems, donor-site seroma and delayed healing, need for a vein graft or alternate recipient vessels, injury to the intrathoracic or intraabdominal structures, transfusion, longer hospital stay and recovery, and the fact that the abdomen can be used only once.

Nipple-sparing, add: partial or complete nipple necrosis, nipple malposition, loss of nipple sensation, inability to breastfeed, and the small possibility of cancer in retained breast tissue behind the nipple.

Patient-specific, add as applicable: higher infection risk with MRSA history or obesity; higher implant failure with obesity or radiation; thrombotic risk with interruption of anticoagulation; higher flap risk with a thrombotic history; delay of adjuvant chemotherapy if a wound complication occurs; T-junction breakdown with a skin-reducing pattern.

Part 6. Modules (insert where they apply)

M1. BORDERLINE NIPPLE-SPARING

The patient's anatomy raises my concern for nipple preservation, specifically [SN-N () cm, N-IMF () cm, ptosis grade (), prior periareolar scars]. I explained that due to the anatomy the risk of losing part or all of the nipple is higher than usual. I explained that due to positioning her nipples will sit low after reconstruction and will likely need to be repositioned at a later revision stage in order to correct their position. I also explained that if the nipples look poorly

perfused during surgery, then I will either convert them to a free nipple graft or remove them rather than risk a wound problem. Patient verbalized understanding and wishes to attempt nipple preservation with these risks in mind. Oncologic eligibility for nipple preservation rests with Dr. [breast surgeon].

M2. NIPPLE NOT A CANDIDATE

I explained that the nipple cannot be preserved because [tumor involvement / retroareolar location / positive margins / SN-N () cm with grade III ptosis], and that the nipple will be removed with the specimen and reconstructed or tattooed at a later stage. [For pendulous breasts: "I explained that a free nipple graft onto the flap is an alternative, with the expected loss of sensation, projection, and pigment, and the patient (elected / declined) this option."] Patient verbalized understanding.

M3. GRADE III BREAST WITH A FLAP: SKIN PLAN

I explained that because of her degree of ptosis the skin envelope requires a plan of its own. I explained that there are two approaches: a Wise-pattern skin-reducing mastectomy in which the flap skin fills the lower pole, which lifts the breast at the first operation but carries a risk of breakdown at the junction of the incisions, or a skin-sparing mastectomy in which the envelope is left loose and lifted at a later revision stage once the blood supply has recovered. I explained that I will discuss the skin pattern with Dr. [breast surgeon] and that the plan will be finalized with the patient at the preoperative visit. Patient verbalized understanding that a second stage is part of the plan rather than a complication.

M4. DELAYED-IMMEDIATE EXPANDER WHEN RADIATION IS LIKELY

I recommended a tissue expander at the time of mastectomy in order to preserve the skin envelope while radiation is completed, with the definitive decision between an implant and her own tissue made at least 6 months from the completion of radiation. I explained that an implant radiated in this setting fails in roughly one of five patients and develops significant capsular contracture in a large minority, and that conversion to her own tissue at that stage avoids most of that risk. Patient verbalized understanding and prefers this approach to [a flat closure with delayed reconstruction / an immediate flap that would be radiated].

M5. DELAYED RECONSTRUCTION AFTER COMPLETED RADIATION

I explained that the best window for reconstruction with her own tissue is 6 to 12 months from the completion of radiation, once the skin has softened and the acute changes have resolved, and that complications are highest between 3 and 6 months after radiation. I explained that an expander in radiated skin carries a higher risk of failure and would likely require a latissimus flap or fat grafting to help the skin. Patient verbalized understanding and elected [].

M6. CONTRALATERAL PROPHYLACTIC MASTECTOMY

I explained that removal of the contralateral breast does not improve survival from her current cancer, that it removes the need for future surveillance of that breast and can simplify symmetry, and that the decision is hers in consultation with Dr. [breast surgeon]. [If genetics pending: "I explained that a positive genetic result would change this discussion and would add counseling regarding her ovaries."] Patient elected [].

M7. PENDING RESULTS GATE THE DATE

I explained that the surgical date will be set after [outside records, the MRI, genetic testing, the medical oncology consultation] are reviewed, because [the extent of disease decides nipple preservation / the subtype decides whether chemotherapy comes before surgery / the genetic result changes the contralateral discussion]. Patient verbalized understanding.

M8. DCIS CONTINGENCY

I explained that in about one of five patients with DCIS the final pathology shows invasive cancer, that a sentinel node biopsy will be performed at the mastectomy because it cannot be performed afterward, and that if invasive disease or a positive node is found, then chemotherapy or radiation may become part of her treatment and the reconstruction will be sequenced around it. Patient verbalized understanding.

M9. SUBTYPE-DRIVEN SEQUENCING (TRIPLE NEGATIVE OR HER2-POSITIVE)

I explained that for her cancer type chemotherapy [with pembrolizumab / with trastuzumab and pertuzumab] is usually given before surgery, that the mastectomy and reconstruction are planned 4 to 8 weeks after the last cycle once her counts have recovered, and that the final pathology after chemotherapy will determine whether radiation is needed. [For surgery first: "I explained that chemotherapy is expected to start within 30 to 60 days of surgery, and that a wound complication could delay it, which is a risk of immediate reconstruction that she accepts."] Patient verbalized understanding. Systemic therapy decisions are per Dr. [medical oncology].

M10. PATIENT DECLINING RECOMMENDED SYSTEMIC THERAPY

The patient stated that she would like to avoid chemotherapy even if it is recommended. I explained that this decision belongs to her and her medical oncologist, that I encouraged her to keep that consultation, and that the reconstruction plan does not depend on her decision. Patient verbalized understanding.

M11. BRIDGING SENTENCE WHEN PREFERENCE AND PLAN DIFFER

The patient initially asked about [implant reconstruction]. After discussion of [her BMI and implant failure risk / her wish to keep her current size / the expected radiation / the durability of her own tissue], she elected [autologous reconstruction] in order to []. Patient verbalized understanding of the longer recovery and the abdominal scar.

M12. UNILATERAL RECONSTRUCTION AGAINST A NATIVE BREAST

I explained that matching a single reconstructed breast to her native breast is more difficult than a bilateral reconstruction, that her own tissue matches a native breast better over time than an implant, and that a contralateral [reduction / lift / augmentation] at a later stage is likely to be needed for symmetry and is covered under WHCRA. Patient verbalized understanding.

M13. COMORBIDITY LINES

Obesity. I explained that her BMI of [] raises the risk of wound complications with either approach and raises the risk of implant failure specifically, which favors her own tissue if she is otherwise a candidate. *Nicotine.* I explained that she must be free of nicotine in all forms for at least 4 weeks before and 4 weeks after surgery, and that I may test for this before surgery. *Diabetes.* I explained that her hemoglobin A1C must be below 8 before elective surgery, and that she will be referred to her primary physician if it is not. *Anticoagulation.* I explained that her anticoagulation will be held and restarted per written instructions from Dr. [], and that her thrombotic history raises the risk of flap thrombosis, which is why []. *MRSA.* I explained that her history of MRSA raises the risk of implant infection, that she will complete a decolonization protocol before surgery and receive appropriate antibiotics, and that I will have a low threshold to remove the device if infection develops. *Hormone therapy.* I explained that [estrogen-containing medication will be stopped 4 weeks before surgery / tamoxifen may be held for 28 days before a free flap as a decision shared with her oncologist].

Part 7. Sign-off consistency check

Before signing, confirm that the laterality, the mastectomy type, the nipple decision, the reconstruction type, and the staging are the same in the history, the exam, the assessment, and every paragraph of the plan; that every option set aside has a reason; that the adjuvant sentence names the oncologists and the basis for the radiation estimate; that the Caprini score is a number with a plan; that the risk sentence matches the chosen pathway; that no education or consent text remains for a modality the patient is not receiving; and that the three visits (preoperative in person, within 30 days, 90-day photographs) carry dates.